



Sexual Safety Policy

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	All areas - including Inpatient, Outpatient and Community services and Safe Haven	All staff

Policy statement

- RPBG has a responsibility to support patients to adopt practices and behaviours that contribute to their sexual safety within its health services.
- All staff working within RPBG are to become familiar with and adhere to this policy.

For the purpose of this document, patients and consumers across all settings will be referred to as patients.

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Definitions

Consensual sexual activity	Any activity of a sexual nature (e.g. sexual touching, sexual intercourse, anal and oral sex) occurring between people over 16 years who have capacity to consent, after provision of mutual sexual consent ¹ .
Sexual safety	Being and feeling psychologically and physically safe, including being free of, and feeling safe from, behaviour of a sexual nature that is unwanted, or makes another person feel uncomfortable, afraid or unsafe. This includes sexual assault and harassment. It also extends to being spoken to using sexualised language or observing other people behaving in a sexually disinhibited manner, including nakedness and exposure or masturbation, being made to watch or shown pornographic images and lacking privacy and dignity when naked ¹ .
Sexual safety incident	A range of sexual acts and behaviours committed against a person without their consent, including: • Sexual assault (a crime of violence) – e.g. any unwanted sexual act or behaviour which is threatening, violent, forced or coercive and to which a person has not given consent or was not able to give consent.² (e.g. unconscious, asleep, intoxicated, having a developmental disability or mental health issues significantly impairing decision making) • Harassment – e.g. unwanted or unwelcome sexual behaviour where a reasonable person would have anticipated the possibility that the person harassed would feel offended, humiliated or intimidated³ • Being spoken to using sexualised language • Sexually disinhibited behaviour and/or observing other people behaving in a sexually disinhibited manner e.g. nakedness and exposure or masturbation, being made to watch or shown pornographic images.¹ Noting, consensual sexual activity within an inappropriate context or setting, e.g. any public or private areas within the health setting.
Sexually disinhibited behaviour	Behaviour of a sexual nature where sexual thoughts, impulses or needs are expressed in a direct or disinhibited way, such as in inappropriate situations; at the wrong time, in the wrong setting, or with the wrong person. (from EMHS policy – Sexual Safety in Mental Health). This behaviour can arise for a variety of reasons and can be a distressing symptom of acquired brain injury, some medications, stroke and other neurocognitive disorders such as dementia and delirium. (from Sexual Disinhibition Psychology Today)

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Version: **3**
 Endorsed by: **Comprehensive Care Committee 23/02/2026**
 Authorised by: **Clinical Practice Committee 19/03/2026**

Date Compiled: **03/2018**
 Executive Sponsor: **Mental Health Division Co-Director**
 Revision Due: **03/2028**

Policy directives

- RPBG has a responsibility to:
 - Encourage and model positive relationships and mutual respect between staff, staff and patients, staff and patients' significant others, and between patients themselves and adopt practices and behaviours that ensure they feel sexually safe
 - Support patients to express themselves in accordance with their gender and/or sexuality and/or bodily diversity in an appropriate context
 - Deliver services:
 - Free from abuse, exploitation, discrimination, coercion, harassment and neglect
 - In an environment in which clear information is shared and is readily accessible for patients, families and advocates regarding individual rights and responsibilities, acceptable and unacceptable behaviours, and the systems of appropriate avenues for complaint, recourse and redress regarding issues of sexual safety
 - Foster a compassionate, sensitive and open culture which encourages the reporting of incidents relating to the sexual safety of patients, their significant others and staff – and where concerns raised will be taken seriously
 - Take appropriate action to prevent and appropriately respond to sexual safety incidents
 - Support trauma-informed care (which assumes an individual is more likely than not to have a history of trauma) as part of universal and targeted approaches to promoting sexual safety ^{1, 4}
- All services are to identify sexual safety practices, standards and/or policies appropriate for their particular setting, in collaboration with all members of the service including staff, patients and their significant others, clinicians, community and advocates where possible.

- Any sexual activity between patients or between patients and their visitors or others on the premises of an inpatient, outpatient or a community-based setting is inappropriate. This includes masturbation in any public area including shared bedrooms, bathrooms and living areas
- Any sexual activity between staff and patients is inappropriate. Refer to **WA Department of Health Code of Conduct** and **WA Criminal Code**
- Intimate and sexual relationships between patients in **residential settings and long-term inpatient or treatment** settings requires careful examination in the context of informed consent, impaired judgement and vulnerability, and availability of privacy to pursue a sexual relationship, as well as the impact on others.

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Policy directives cont'd

- Refer to the following documents where necessary:
 - **Sexual Safety: Responding to Incidents Policy**
 - **ED Presentation of Alleged Sexual Assault Management SOP**
 - **Recognising and Responding to Family and Domestic Violence and Elder Abuse SOP**
 - **Assessing Patient Capacity SOP**
 - WA Department of Health policies:
 - **Procedure for Responding to a Recent Sexual Assault**
 - **Responding to an Allegation of Sexual Assault Disclosed within a Public Mental Health Service Policy**
 - **Child and Adolescent Health Service (CAHS) Guidelines for Protecting Children**
 - **Consent to Treatment Policy.**

Additionally, Mental Health Division (MHD) services are to:

- Practice within the
 - **EMHS Sexual Safety in Mental Health Policy**
 - [Chief Psychiatrist's Guidelines for the Sexual Safety of Consumers of Mental Health Services in Western Australia](#)
- Promote an understanding of practices that support sexual safety during multidisciplinary meetings, clinical handover/rounding or other staff forums
- Ensure staff complete sexual safety training (see [Staff training and education](#) section)
- Promote referral pathways to support staff requiring assistance with trauma
- Prominently display information on sexual safety expectations on a notice board or in an area accessible to all patients, significant others and others accessing the service
- Consider use of relevant posters/info sheets
- For MHD inpatient services – refer to available Sexual Safety Expectations Poster, Staff to Consumer Sexual Safety Discussion Prompt Sheet and note all MH services may refer to and use the [Chief Psychiatrists Sexual Safety Guidelines](#) or create relevant information for patients and significant others.

Sexually safe environment criteria

- Trauma-informed, and emphasises recovery^{1, 4}
- Balances personal autonomy and decision making with a duty of care to provide a safe and therapeutic environment for all patients, significant others, visitors, staff and the related community
- Recognises the common need of patients for privacy and personal space, and that these needs vary between individuals regardless of gender, sexual or cultural identity
- Recognises the need for routine assessment, treatment, early recovery
- Actively manages potential disruptions to treatment and wellbeing as a result of sexual activity and/or sexual safety incidents and/or vulnerability for all patients
- Promotes personal self-care, respect, resilience and self-determination⁵
- Services should ensure the physical environment supports sexual safety for all patients, particularly vulnerable cohorts. Refer to the [Australasian Health Facility Guidelines](#) to take account of sexual safety⁶ and the [Chief Psychiatrist's Guidelines for the Sexual Safety of Consumers of Mental Health Services in Western Australia](#) (where appropriate).

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Impacts of sexual abuse, assault, harassment or inappropriate sexual activity

- The relationship between childhood abuse and the development of MH problems is well known⁵
- All clinicians should be aware of possible past sexual abuse and how this may increase a patient's vulnerability to further inappropriate sexual activity, exposure or exploitation by others, impact on their presentation, interaction with others and vulnerability
- Refer to the [Sexual Assault Resource Centre \(SARC\)](#) website for information and resources.

Recognising patients with increased sexual safety vulnerability

Risk factors of sexual vulnerability¹

- Sex recorded at birth as female
- Aboriginal person
- CaLD background (culturally and linguistically diverse)
- Identifying as part of the LGBTIQ+ community
- Young person / minor
- Older person
- Capacity – including ability to provide sexual consent and understand tactics of coercion
- Impaired cognitive function
- Sexual trauma
- Childhood trauma
- Refugee/torture survivor
- Victim or survivor of family and/or domestic violence
- Social isolation
- History of sexual offending
- Impaired communication skills
- High levels of distress
- Co-occurring alcohol and/or other drug use
- Mental illness / experiencing psychosis
- High levels of sedation
- Sexual disinhibition.

Assessment of sexual safety and sexual vulnerability

- Assess and document the vulnerability of each person on their admission to the service in the RPBG Multidisciplinary Care Plan, including any history of sexual safety incidents or incidences of sexual disinhibition or impulsive behaviours
- Reassess as appropriate in alignment with the Risk Re-assessment record in the *RPBG Multidisciplinary Care Plan*
- Apply principles of trauma-informed care
- Determine whether the patient is exhibiting any potentially related signs or symptoms and meets any known criteria for increased vulnerability (as above) from current presentation, observation and developmental history taking
- Organise a private space and adequate timeframe to enable the patient to proceed at their own pace
- Use interpreters, Aboriginal workers, traditional healers, elders, and/or advocates, as appropriate and with consideration as to whether this assisting person is known to the patient or the patient's community
- Take an appropriate history of previous trauma (e.g. sexual trauma), where relevant
 - Ask in the context of general psychosocial history (not at the outset of an assessment)
- Approach relevant questions and discussion topics employing clinical judgement and using a format and appropriate language to encourage the patient to feel comfortable to talk – noting explicit discussions regarding sex and masturbation may not be appropriate for all patients
- Consider that some patients may have a history of an intimate nature with other patients
 - Where identified, management is required and the potential for alternative care arrangements
- Consider the patient's potential to assault or exploit others and implement interventions to mitigate the risk where possible
- Consider the patient's cultural and religious background and how this may impact their feelings of vulnerability and sexual safety on the ward
- Assess the potential risk to the patient and others from any impulsive or disinhibited behaviour. Where appropriate and with consent, consult with family or carers to gather relevant history and identify any risk factors related to sexual disinhibition or vulnerability in the home or community setting including domestic violence
- With patient consent, consult with significant others and services already involved in formulating a patient's *What Matters Most (WMM) Record* and/or crisis awareness plan
- Patients identified at risk will be supported to access education to enable them to effectively recognise and respond to behaviours, both their own and other people, that may compromise or breach their own or another person's sexual safety
- Document in patient's healthcare record
 - For MH areas, assessment will inform the *Risk Assessment Management Plan (RAMP)* and use of *Progressive Risk Assessment (PRA)* (inpatients only)
 - For general health areas, assessment will inform the *Behavioural Observation Form (BoF) RPH*.

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Assessment of sexual safety and sexual vulnerability cont'd

Where sexual safety assessment feels out of scope of practice, staff are to escalate to line manager and consider accessing MH team/Consultation-Liaison Psychiatry Service/ Behaviour Support Team (BeST) +/- Safety For Everyone (SAFE) at Royal Perth Hospital [RPH] or BeSAFE at Bentley Health Service [BHS] for support.

Supporting sexual safety

- Apply principles of trauma-informed care. Proactively identifying potential for re-traumatising patients with trauma histories ^{7,8}
- During each episode of care and at multidisciplinary team (MDT) meetings, ensure that at-risk patients are clearly identified in clinical handovers. This includes highlighting any sexual safety concerns identified during the initial assessment (e.g., via RAMP in mental health settings or BoF in general health settings) and documented in the patient's healthcare record. Include specific plans for monitoring and managing any sexually disinhibited behaviour
- Where a patient's vulnerability is largely a function of their mental illness and/or decline in cognitive function, maintain an active approach to the assertive treatment of acute symptoms and distressed behaviours
- Consider and determine patient preference for staff gender - noting allocation will be impacted by staff availability, clinical need and service acuity
- Set appropriate alerts as relevant (e.g. on iClinical Manager, Clinical Information Sysytem [CIS], Psychiatric Services Online Information System [PSOLIS]), WebPSOLIS), activate patient of concern alert on iSoBAR information handover, an alert on the patient's journey board to highlight and communicate risks
- Increase staff vigilance and/or patient observations (inpatient areas only) during admission and at times of transition, as appropriate. Refer to the **Special and Supportive Observations Policy**
- Choose therapy rooms with an observation window and/or keep the door ajar during sessions where relevant
- Be cognisant that medical consultations, examinations, procedures and investigations – particularly when intimate, have the potential to cause the patient distress and could be misinterpreted and lead to allegations of inappropriate staff conduct
 - Adhere to and apply the basic principles of respect, explanation, consent and privacy
 - Consider the presence of a companion, PSP or chaperone. Refer to the **EMHS Chaperone Policy**
- Consider sexual safety risk as a trigger for escalation of care. Refer to **Recognising and Responding to Acute Deterioration (RRAD) in Mental State, Cognition and Behaviour SOP** and **RRAD Policy**
- Where a risk of sexual assault or domestic violence is identified, refer to Social Work and Alcohol and Other Drugs Services where relevant to discuss options for support and crisis plans. Refer to **R PBG Recognising and Responding to Family and Domestic Violence and Elder Abuse SOP** and **WA Department of Health Responding to the Abuse of Older People (Elder Abuse) Policy**

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Supporting sexual safety cont'd

- Inform patient of suppression of healthcare record and/or periodically revisit a patient's wishes regarding this, where relevant risks have been flagged
 - Patients may make such a request to admitting clinician or Treating team who will advise
 - RPH: nursing team contacts Bed Management System Officer (BMSO) 1 or 2 or after hours: Operations Hub Clinical Nurse Manager to activate and document suppression
 - BHS or Transitional care Unit (TCU): relevant MH Coordinator of Nursing (CoN), Shift Coordinator or A/H BHS MH Duty CNS to activate and document suppression
 - Enter on WebPAS
 - Request ward clerk to record into WebPAS for staff with no access
- Document any issues arising and how they have been or are being managed in the patient's healthcare record and/or on relevant *Patient Care Plan (PCP)*, and/or *Treatment, Support and Discharge Plan (TSDP)* on WebPSOLIS and handover documentation to reflect the identified risk and management.

The following patient education and engagement is also applicable to MH services:

- All patients are to be provided with an introduction to the relevant unit and/or service which will include a verbal explanation relating to sexual safety, including:
 - The rights of all patients to a safe and therapeutic environment
 - Mutual responsibilities for staff and patients
 - That sexual activity on the unit or service is inappropriate and not permitted
 - That intimate behaviour or sexualised contact with other patients, visitors or staff is not acceptable
 - That patients are not permitted in other patient's single rooms
- Collaborative care and safety planning¹
 - Ensure completion of relevant *WMM form* and/or crisis awareness plan, *RAMP/BoF* and care plans, including specific triggers, circumstances and risk mitigation plans
 - Encourage the patient's involvement in activities and programs to minimise isolation and vulnerability where applicable
 - Familiarise patients with their rights, responsibilities and acceptable use regarding recording and sharing of images of themselves and others online and on social media sites, use of personal electronic devices and/or online resources. Refer to **Personal Electronic Communication Devices for Mental Health Inpatient Consumers Policy**
 - Promote the patient's positive and respectful approach to their sexuality, gender identity and to sexual relationships (e.g. how to assert their right to say 'no,' including to their partner or spouse), and how to access relevant education and referral services.

All inpatient services

- For patients particularly vulnerable in regard to their sexual safety, consider the following, including staffing, bed and room allocation options for inpatient services where possible and/or appropriate
 - Endeavour for maximum visibility for patients deemed at risk. Vulnerable and sexually disinhibited patients are to be allocated beds and bedrooms closest to the staff base or reception if possible and specific access to toilet and bathroom facilities that may be required
 - Consider increasing visual observations if appropriate. Refer to the **Special and Supportive Observations Policy**
 - **Sexually disinhibited patients should not be allocated to a mixed gender room**
 - If unable to meet these requirements, escalate to Shift Coordinator or local manager and document
 - Provide patients who feel vulnerable and are distressed by the lack of privacy and dignity with single or same gender rooms where practicable
 - Update bed request and/or allocation on Enterprise Bed Management (EBM) programme and/or liaise with OHC or After Hours Duty MH CNS for same gender room or single room
 - Access to lockable bedroom doors, where possible
 - Ensure accessible areas (e.g. lounge or communal areas, etc.) are visible by staff, where possible
- Inpatient services should have 2 staff, ideally one of the same gender or preference as identified by the patient, are to be present throughout the night shift and if required, based on the medical assessment and risk assessment, hourly rounding or hourly night-time visual observations to occur where practicable
- Monitor visitor interactions for inappropriate behaviour, particularly of a sexual nature.

Recognising sexual safety offending behaviour

- RPBG staff should be aware of potential offender behaviours in patients as well as those that care for them
- Deliberate tactics to perpetrate sexual assault may be used to select and abuse – particularly MH patients, including:
 - Recruiting these patients by means of bribes (money, cigarettes, food, other items) or offering perceptions of care, love and intimacy
 - Utilising power and/or positions of power to gain confidence and trust for the purposes of gaining sexual favours
 - Maintaining this accessibility by using threats to discourage the individual from disclosing the assault
 - Grooming staff and others by ingratiating themselves as helpful and suggesting to staff that the individual they assaulted is lying
- Identify patients who have a history of sexually inappropriate or sexually disinhibited behaviour or sexually abusive and offending behaviours
- Some patients may be potential offenders due to their mental illness or cognitive impairment - e.g. a patient with sexual disinhibition may act out their impulses; others may be more predatory in intent and engage in opportunistic behaviour, targeting vulnerable patients, which may be a repetition of their own experience.

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Management of identified potential offenders

- Identified individuals to be closely monitored and regularly assessed throughout their patient journey – noting times of transition may increase risks
- Collaborate with patient and, with consent, significant others regarding patient care plan documentation including risk assessment and discharge documentation when appropriate
- Additional assessment should be included in the relevant *PCP* and/or *TSDP* documentation (including other management plans and *RAMP* for MH areas and *BOF* for general health), and patient's healthcare record to include history and reports of:
 - Sexual disinhibition or aggression from whatever cause – e.g. mental illness, substance use, acquired brain injury
 - Sexual activity in previous admissions or having a history of inappropriate sexual activity or sexually offending behaviour
 - Past sexual abuse perpetrated against them
 - Violent behaviour, threats or intimidation
 - Domestic violence
 - Illicit drug and alcohol use

Note: Risk alerts are scanned into the healthcare record (MHD services) and/or printed and placed in 'bedside' file and documented on PSOLIS/webPSOLIS, iClinical Manager, WebPAS/ Mastercare

- Risks are to be discussed at review meetings, MDT review meetings and handover
- Identified patients, where necessary, are to be offered discreet, non-intrusive and non-custodial safety measures such as a temporary physical location that provides safety for all
- Ensure relevant discharge documentation accurately and sensitively reflects any alleged or actual sexual safety breach(es), or sexual disinhibition. Include plan for future admissions and an outline of necessary support and referral for the patient and their significant others.

For staff response to a sexual safety incident. Refer to **R PBG Sexual Safety: Responding to Incidents Policy**.

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Supporting staff safety

- All RPBG staff have the right to feel safe from acts that compromise or breach their sexual safety while in a health service environment or during their work in the community and will be supported appropriately
- When assessing risk to staff, clinical teams and individual staff should consider:
 - Any patient history of false allegation or complaints of a sexual nature
 - Current mental state and presentation, such as mania, psychosis, intoxication, or personality traits associated with sexual risk
 - Documented alerts which identify concerns regarding sexual safety of staff for patients (e.g. on PSOLIS/webPSOLIS, iClinical Manager)
 - Any patient history of boundary violations with previous staff.

Staff are to:

- Raise any concerns with their line manager or senior staff member if they believe a patient's behaviour is placing them at risk
- Take care not to place themselves in vulnerable settings whereby the potential for allegation may arise such as being isolated in patient bedrooms
- Dress in a manner appropriate to their professional role and in accordance with the **EMHS Dress Code – Staff, Contractors and Volunteers Standards of Clothing Policy**.

Also refer to **RPBG Sexual Safety: Responding to Incidents Policy**

Staff training and education

- All clinical staff must complete Mandatory Reporting of Child Sexual Abuse with a 3 yearly refresher as per EMHS Mandatory Training Matrix
- Mental Health Division frontline staff, managers and support workers are to be provided with training to enable them to minimise sexual safety incidents, and work to prevent and respond effectively to these. Refer to **EMHS Sexual Safety in Mental Health Policy, WA Department of Health Code of Conduct** and RPBG in-service sexual safety training
- All other areas: a variety of education is available to support trauma informed care and awareness of sexual safety risk and management. Managers to use discretion to facilitate staff training (including peer workers, volunteers)
- For volunteers, refer to **EMHS Volunteer Policy** for guidance about rights and responsibilities, the **WA Department of Health Code of Conduct** and how to respond if they receive a disclosure of sexual assault or become aware of a sexual safety issue
- Agency staff are to receive routine orientation to assist in managing sexual safety where relevant to assigned patients. Refer to Line Manager/Shift Coordinator/Team Leader to handover identified risks.

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Compliance monitoring

- RPBG management are to ensure that staff members and volunteers adhere to the policy and related procedures
- For volunteers, refer to **EMHS Volunteer Policy** and the **WA Department of Health Code of Conduct**
- Monitoring of Datix CIMS related to patients' sexual safety and/or number of incidents reported to the Chief Psychiatrist
- Mental Health Division to perform environmental audit to monitor and support sexual safety for mental health authorised inpatient services in alignment with Chief Psychiatrist requirements as per the [Chief Psychiatrist's Standards for Authorisation of Hospitals under the Mental Health Act 2014](#) (Appendix B).

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Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Sexual Safety Policy through consultation with the Aboriginal Health Strategy Team and Aboriginal Workforce Engagement Group to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 3006).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

Related policies, practice standards, clinical guidelines

Policies and procedures SharePoint

- [LGBTIQ+ Inclusivity for Patients Policy](#)
- [Clinical Handover Policy](#)
- [Progressive Risk Assessment \(PRA\) SOP](#)
- [Safe Haven Access and Provision of Care SOP](#)
- [ED Presentation of Alleged Sexual Assault Management SOP](#)
- [Sexual Safety: Responding to Incidents Policy](#)
- [Assessing Patient Capacity SOP](#)
- [Special and Supportive Observations Policy](#)
- [Recognising and Responding to Acute Deterioration \(RRAD\) in Mental State, Cognition and Behaviour SOP](#)
- [RRAD Policy](#)
- [Personal Electronic Communication Devices for Mental Health Inpatient Consumers Policy](#)

EMHS Policies

- [Sexual Safety in Mental Health Policy](#)
- [Notifications requirements under the WA Mental Health Act 2014 Policy](#)
- [Chaperone Policy](#)
- [Dress Code – Staff, Contractors and Volunteers Standards of Clothing Policy](#)
- [Volunteer Policy](#)
- [Mandatory Training Policy](#)

WA Department of Health Policy Hub

- [Clinical Incident Management \(CIM\) Policy](#)
- [Discipline Policy](#)
- [Notifiable and Reportable Conduct Policy](#)
- [Complaints Management Policy](#)
- [Statewide Standardised Clinical Documentation \(SSCD\) Guideline](#)
- [Procedure for Responding to a Recent Sexual Assault](#)
- [Responding to a Recent Sexual Assault: Supporting Information](#)
- [Consent to Treatment Policy](#)
- [Code of Conduct Policy](#)
- [Responding to the Abuse of Older People \(Elder Abuse\) Policy](#)

Other

- [Guidelines for Protecting Children – Child and Adolescent Health Service](#)
- [WA Chief Psychiatrist's Standards for Clinical Care 2015](#)
- [WA Chief Psychiatrist's Guidelines for the Sexual Safety of Consumers of Mental Health Services in Western Australia](#)
- [Responding to an Allegation of Sexual Assault Disclosed within a Public Mental Health Service Policy – Office of the Chief Psychiatrist](#)
- [Charter of Mental Health Care Principles \(MHA 2014\) – Mental Health Commission](#)
- [Mental Health Act \(MHA\) 2014](#)
- [WA's first Sexual Violence Prevention and Response Strategy launched](#)
- [Criminal Code Act 1913](#)
- [Criminal Investigations Act 2006](#)
- [Guardianship and Administration Act 1990](#)
- [Health Practitioner Regulation National Law \(WA\) Act 2010](#)

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Related forms

- *Behaviour Observation Form (BOF)*
- *What Matters Most Record*

Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021);

Standard 1: Clinical Governance;

Standard 2: Partnering with Consumers;

Standard 6: Communicating for Safety

NSMHS Standards (2010): Standard 2: Safety; Standard 5: Consumers; Standard 8: Governance, leadership and management

Australian Charter of Healthcare Rights Guiding Principles: Safety and Respect.

References

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